

Outpatient Medical Record

Department: Emergency Department	Outpatient No.: 24152017
Name: Chenbin Feng	Contact: 13867294531
Gender: Male	Age: 42 Marital Status: Married

2026-02-27 16:41 — Initial Visit Record

Chief Complaint: Neck pain following traffic accident injury.

Present Illness: The patient was involved in a rear-end collision while riding in a passenger vehicle approximately 3 hours ago. Following the injury, he developed neck pain and stiffness that was significantly aggravated with neck rotation. Consciousness remained clear throughout; no loss of consciousness, nausea, vomiting, or limb weakness was reported. The patient was brought to the emergency department accompanied by family members.

Past Medical History: History of cervical spondylosis with intermittent neck and shoulder soreness.

Allergy History: None.

Physical Examination:

1. Tenderness at the posterior cervical midline and bilateral paravertebral regions (+); restricted cervical range of motion with marked limitation in flexion/extension and rotation.
2. Bilateral upper limb muscle strength grade 5; no significant sensory deficit; no pathological reflexes elicited; vital signs stable.

Laboratory / Auxiliary Examinations:

1. Cervical CT: No definite fracture or dislocation; straightening of the cervical physiological curvature.
2. Cervical MRI: Mild disc herniation at C5/6 and C6/7; posterior cervical soft tissue edema signal.

Preliminary Diagnosis:

1. Cervical soft tissue injury (whiplash injury)
2. Cervical spondylosis (mild disc herniation at C5/6 and C6/7)

Management Plan:

1. Condition stabilized after emergency treatment; analgesic, muscle relaxant, and neurotrophic symptomatic supportive therapy administered.
2. Short-term cervical collar protection recommended; advised to avoid prolonged forward head posture, sudden neck rotation, and heavy lifting.
3. Instructed on hot compress application and gentle cervical rehabilitation exercises; gradual resumption of activity as symptoms improve.

4. Follow-up visits recommended at 2 weeks and 6 weeks; repeat cervical MRI if necessary to assess recovery.
5. Medical leave of approximately **12 weeks** recommended, with continuous cervical functional rehabilitation training.

Physician Signature: _____

Reminder: Please bring this medical record at every follow-up visit. Thank you!